

Crisis-Response Protocol — Milton Nation (DEFAULT DRAFT for clinical review)

Status: DRAFT for the Clinical Director to review, adjust, and sign. This is a sensible default written by the build team — **not** final clinical guidance. Everything marked **[SET BY CLINICAL]** must be decided by qualified clinical staff. **The app cannot onboard real patients until this is completed and signed (Section 6).**

1. What this app is (and isn't) — the safety framing

Milton Nation is a **peer-support + resource-navigation** app for recovery alumni. It is **NOT** an emergency service, a crisis hotline, or a 24/7-monitored clinical system. This must be stated to users **in-app** (onboarding + the crisis sheet):

*"Milton Nation is a peer-support community, not an emergency service, and is not monitored 24/7. If you are in danger or crisis right now, call or text **988** (Suicide & Crisis Lifeline) or **911**."*

2. What the app already does automatically

- Scans posts/messages for crisis content → holds the post and creates a **content flag** (`content_flags` , risk level low/medium/high).
- Shows the in-app **crisis resources sheet** (988, Crisis Text Line 741741, SAMHSA, Milton FL/OH lines, 911).
- On flag: sends a **push to all admins + super-admins** and drops it in the **Content Flags admin queue**.
- "I'm Struggling" button surfaces resources + can notify the care team.

This protocol defines what the HUMANS do next.

3. Who responds (the human chain) — [SET BY CLINICAL]

Role	Responsibility
Primary responder	[SET: e.g., the alumnus's assigned case manager] — reviews the flag, reaches out to the member.
Backup responder	[SET: e.g., on-call therapist / facility admin] — covers if primary is unavailable.

Role	Responsibility
Clinical escalation	[SET: Clinical Director or designee] — decision-maker for imminent-risk situations.
Facility routing	FL flags → FL care team; OH flags → OH care team (the app already separates by facility).

4. Response times + coverage hours — [SET BY CLINICAL]

- **Coverage hours:** [SET: e.g., Mon–Fri 9:00am–5:00pm ET]. The app is **NOT** monitored outside these hours.
- **Acknowledge a HIGH-risk flag within:** [SET: e.g., 1 hour during coverage hours].
- **Acknowledge a medium/low flag within:** [SET: e.g., 1 business day].
- **After-hours / no coverage:** the in-app crisis sheet (988/911) is the safety net; a human follows up [SET: next business morning]. The after-hours disclaimer in Section 1 must be visible so no one believes a human is watching in real time.

5. Escalation ladder — [SET BY CLINICAL]

1. **Low / medium risk:** responder checks in with the member via in-app care-team chat within the SLA; documents the contact.
2. **High risk (not imminent):** responder contacts the member directly [SET: call / secure message] and loops in clinical escalation.
3. **Imminent danger** (stated intent + means/plan, or "going to use/hurt myself tonight"): [SET clinical action — e.g., attempt direct contact; if unreachable and risk is imminent, contact the member's emergency contact and/or initiate a welfare check via 911 per clinical judgment]. Clinical Director (or designee) owns this call.

6. Documentation + sign-off

- Every crisis response is logged: the app records the flag in `content_flags` and access in `emergency_access_log` / `audit_logs`; the responder documents clinical follow-up [SET: in the member's treatment record].
- **Review cadence:** revisit this protocol [SET: quarterly].

Clinical sign-off (required before launch):

I have reviewed and approved this crisis-response protocol.

Name: _____ Title: _____

Signature: _____ Date: _____

7. Build-team to-do once clinical fills this in

- Add the after-hours/not-24-7 disclaimer to onboarding + the crisis sheet (Section 1 wording) — **Claude can implement in ~15 min once approved.**
- Confirm the crisis resource phone numbers in-app are current (988, SAMHSA 1-800-662-4357, Milton FL (844) 406-4325, Milton OH (740) 715-4673).
- Verify the flag → correct-facility care-team routing matches the roles set in Section 3.